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Name: حليمه عبدالحكيم محمد عبد الجليل National ID: 26411150101784	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريرنا عن حالته
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تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد انه
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A patient of lymphoma for Supportive and palliative Treatment and blood transfusion/blood products
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التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

جمهورية مصر العربية
بطاقة تحقيق الشخصية



خليله
عبد الحكيم محمد عبد الجليل
٣٩ جمال عبد الناصر - المعصرة - عرب سلام
المعصرة - القاهرة

١٩٦٤/١٠/١٥ ٢٦٤ ١١ ١٥ ٠١ ٠١٧ ٨٤

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٢٠٢٠/٠٣ ٢٦٤ ١١ ١٥ ٠١ ٠١٧ ٨٤

ربة منزل

متزوجة
مسلمة
أنثى
حسانى عبد الحى محمد عبد الجليل
البطاقة سارية حتى ٢٠٢٧/٠٣/٠٨





Dr.
Soliman Saba
M.Sc., M.D. Pathology
Prof. Of Pathology
Faculty of Medicine, Cairo University
Fellow, Ohio State University U.S.A.

دكتور
سليمان سابا
أستاذ الباثولوجيا
كلية طب قصر العيني
زميل جامعة أوهايو بالولايات المتحدة الأمريكية

Name: **حليمه عبد الحكيم محمد**

Age: 61 years

Clinical Diagnosis:

Nature of specimen: Laparoscopic cholecystectomy.

Referred By: Prof. Dr. WAHEED HALIM.

Date: 14/05/2025

PATHOLOGY REPORT

GROSS: Specimen received consists of a gall bladder measuring 7.5x3.5x2 cm and containing three combined stones; the largest measuring 1.5x1.5x1 cm. The wall of the gall bladder (at the fundal region) showed a whitish firm tumour mass; measuring 3.5x2.2 cm. Tumour tissue was seen infiltrating the whole thickness of the gall bladder wall.



MICROSCOPIC: Serial sections examined from the specimen received revealed a thickened gall bladder wall showing an infiltrating fundal tumour tissue formed of irregular glands, papillae and cribriform structures lined by malignant columnar epithelial cells showing high nucleo/cytoplasmic ratio and hyperchromatic or vesicular nuclei with prominent nucleoli and scattered mitotic figures. The stroma separating tumour glands is moderately desmoplastic. Tumour tissue was seen infiltrating the whole thickness of the gall bladder wall; including the mucosa (with ulceration) and muscle layer as well as perimuscular connective tissue. The serosal covering is free from infiltration by tumour tissue. Few lymphatic tumour emboli were detected. Surgical margins at the cystic duct are free. No evidence of blood vessel invasion by tumour tissue.

DIAGNOSIS: Gall bladder, laparoscopic cholecystectomy, Adenocarcinoma, Grade II, T2, infiltrating The Whole Thickness Of The Gall Bladder Wall And Surrounding Perimuscular Connective Tissue. Free Surgical Margins.

Thanks for reference,

Soliman Saba
14/5/2025

Name : Halima Abdulhakim Mohammed
Branch : Maadi
Ref.Physician Prof : Ahmed El Khateb

Date : 12-06-2025 11:00
Medical No. : 125659

- The urinary bladder shows normal size and wall thickness with no calculi inside.
- Normal C.T. appearance of the uterus .
- Normal ischeo-rectal fossae on both sides.
- No other sizable abdominal or pelvic lymph glands.
- Scanned lung bases appear clear.
- No suspicious bony lesions.

OPINION:

- Diffuse fatty hepatomegaly for correlation with hepatic and lipid profile.
- A large porta hepatis lymph node as forementioned, for correlation with previous studies .

Much Obliged,
Dr.Mona Ali,MD

Scanned with

Name : Halima Abdulhakim Mohammed
Branch : Maadi
Ref.Physician Prof : Ahmed El Khateb

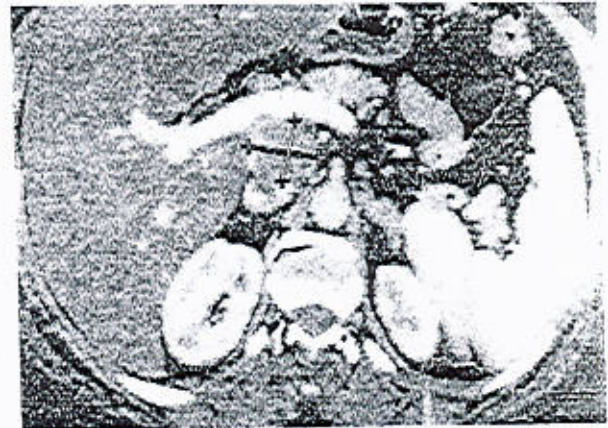
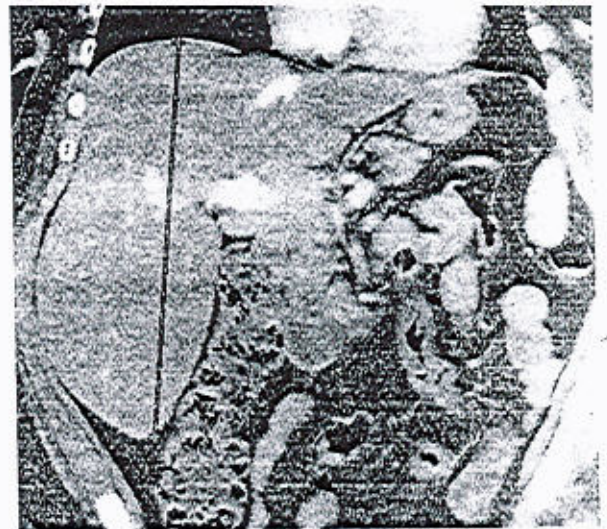
Date : 12-06-2025 11:00
Medical No. : 125659

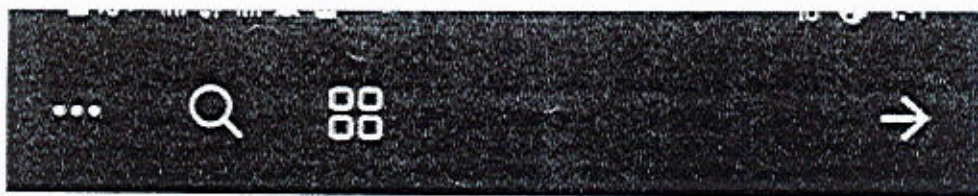
TRIPHASIC CT SCAN OF THE PELVIABDOMEN

TECHNIQUE: Contiguous helical axial images of the abdomen were obtained from the base of the lungs down to the iliac crest after administration of intravenous contrast (as per liver protocol).

FINDINGS:

- *The liver is enlarged reaching 22 cm showing diffuse decreased hepatic attenuation denoting diffuse fatty hepatomegaly. No definite hepatic focal lesions detected throughout the different contrast phases. No dilated IHBRs.*
- The portal vein is patent.
- *The gall bladder is surgically removed with surgical clips at the gall bladder fossa.*
- *A large porta hepatis lymph node is seen measuring 29 x 36 mm seen indenting the IVC with no clear fat planes .*
- *Bulky left adrenal gland yet with no masses seen .*
- The spleen is of average size with no focal lesions.
- The pancreas, the right adrenal gland and both kidneys are within normal limits.





23 NILE

11/05/2025 02:00

Name : Hadeema Abdel Hakeem Mohamed

Date : 03-05-2025 02:00

Abd Al Galeel

Branch : Maadi

Medical No. : 110870212

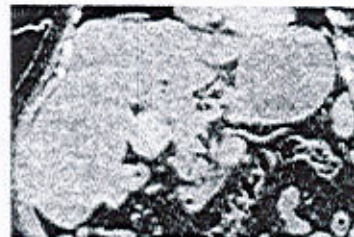
Ref.Physician Prof : وحيده حليم

Ins.Company : 2025

CT SCAN OF THE ABDOMEN WITHOUT CONTRAST

FINDINGS:

- The liver is enlarged (19.5 cm) with diffuse decrease hepatic attenuation yet with no definite HFLs seen.
- The gall bladder is well distended with two radiodense stones of average size 16mm.
- Few left subcentimetric para-aortic lymph nodes
- The spleen, the pancreas, adrenal glands and both kidneys are within normal limits.
- The abdominal aorta is of normal diameter. There is no evidence of abdominal aortic aneurysm.
- The visualized portion of the bowel is unremarkable.
- There is no evidence of free intraperitoneal fluid or air.



IMPRESSION:

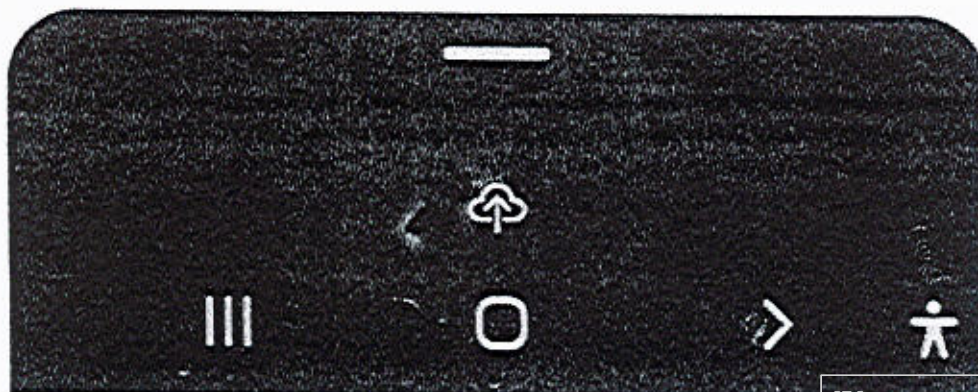
- Diffuse fatty hepatomegaly for correlation with lipid profile.
- Calcular gall bladder disease .Post -contrast study is recommended for assessment of any gall bladder masses..

Much Obliged,

Mona Ali,MD

Dr. Mona Ali

11/05/2025 02:00



Name : Halima Abd Elhakeem

Date : 02/05/2025

ABDOMEN AND PELVIC ULTRASONOGRAPHIC SCANNING
REVEALED:

- Liver is of enlarged sized measuring 19.7 cm in RT MCL. Parenchyma displays bright homogenous echopattern. No focal lesions or intrahepatic biliary dilatation are seen. **Portal vein:** is patent.
- Gall bladder is partially contracted with multiple large stones and **fundal focal intraluminal ill-defined heterogeneous isoechoic soft tissue mass lesion 3.8x3 cm** with hyperechoic foci within the mass.
- Spleen is average sized and showing homogenous echopattern, measuring 10.6 cm in its long axis.
- Pancreas and para-aortic region are free.
- Both kidneys are of average size and shows homogenous echopattern. No back pressure changes. No sizable stones or masses are seen.
- Uterus is of average size showing no masses or cysts.
- No adnexal solid or cystic lesions.
- The urinary bladder shows normal wall thickness and clear contents. No stones, polyps or masses, or diverticular.
- No ascites.

CONCLUSION:-

- GB stones with suspected intraluminal fundal mass lesion for CT abdomen with contrast
- Fatty liver
- Colonic gaseous distension

Much obliged,

Dr. Omnia Bakry MS c